BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Safety and response tests have different purposes. Use the individual prescription and appointment dates.

Check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. Review symptoms, PSA and testosterone; scans and the frequency of tests follow the individual plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional prepares and gives the injection using the product-specific instructions. Confirm the 11.25 mg formulation, agreed route and next appointment. Other triptorelin strengths have different schedules.	Keep an accurate medicine / injection record.	The team assesses temporary anti-androgen protection against the initial hormone flare. Report worsening pain or trouble passing urine promptly. New weakness or changes in bladder/ bowel control require emergency assessment. Keep the written injection dates even if you feel well.	Continue until the team changes the plan.
Cancer response	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00480 v3: official regimen · Decapeptyl 3-month: product information

Tests in your plan

Blood count, kidney and liver tests

WHEN

Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. Review symptoms, PSA and testosterone; scans and the frequency of tests follow the individual plan.

Checks blood cells and organ health.

Blood glucose and bone blood profile

WHEN

Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. Review symptoms, PSA and testosterone; scans and the frequency of tests follow the individual plan.

Checks metabolic effects and blood minerals.

PSA and testosterone

If part of your plan

WHEN

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

Checks cancer activity and hormone suppression.

electrocardiogram (ECG) and bone-density assessment

If part of your plan

WHEN

ECG (heart tracing) and bone-density scans are arranged according to heart-rhythm risk, fracture risk and clinical need. Your team confirms which tests you need.

Checks heart rhythm or fracture risk when indicated.

THE WHOLE TREATMENT JOURNEY

Triptorelin 11.25 mg: hormone injection every 12 weeks: treatment and checks

Locally advanced or metastatic hormone-dependent prostate cancer under NCCP 00480.

Triptorelin 11.25 mg is given every 12 weeks (84 days). The usual NCCP route is into a muscle; the source also permits injection under the skin for this formulation.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety NCCP · SMPC	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. Review symptoms, PSA and testosterone; scans and the frequency of tests follow the individual plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments NCCP · SMPC	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional prepares and gives the injection using the product-specific instructions. Confirm the 11.25 mg formulation, agreed route and next appointment. Other triptorelin strengths have different schedules.	Keep an accurate medicine / injection record.	The team assesses temporary anti-androgen protection against the initial hormone flare. Report worsening pain or trouble passing urine promptly. New weakness or changes in bladder/ bowel control require emergency assessment. Keep the written injection dates even if you feel well.	Continue until the team changes the plan.
Cancer response NCCP · SMPC	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

What each check is for

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Treatment and review

One injection every 12 weeks

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Ask / act: Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. Review symptoms, PSA and testosterone; scans and the frequency of tests follow the individual plan.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.52